

Acne Fulminans

Bone pain is common, especially at the clavicle and sternum. Radiologic examination may demonstrate lytic bone lesions. Occasionally, there is accompanying erythema nodosum. Although this disease is often classified with acne conglobata, there are basic differences. The onset of acne fulminans is more explosive, nodules and polymorphous comedones are less common, the face is not involved as frequently, and the neck is usually spared. Ulcerative and crusted lesions are unique, and systemic symptoms are more common.

Systemic glucocorticoid therapy, along with oral antibiotics and intralesional glucocorticoids, is the treatment regimen required for these patients. Isotretinoin is also beneficial, but to prevent explosive flares, systemic glucocorticoids must be started before isotretinoin and continued during the first few weeks of therapy. The initial dose of isotretinoin should be lowered in the first weeks until inflammation is controlled. The daily dose of glucocorticoids should be slowly tapered as tolerated. Dapsone in combination with isotretinoin has been reported beneficial in cases associated with erythema nodosum.

SAPHO Syndrome

SAPHO syndrome is characterized by synovitis, acne, pustulosis, hyperostosis, and osteitis. It is predominantly associated with hyperostosis of the anterior chest, palmoplantar pustulosis, hidradenitis suppurativa, and acne fulminans. Its etiology is unknown.

Reported successful treatments include nonsteroidal anti-inflammatory drugs, sulfasalazine, and infliximab. Bisphosphonates are beneficial for managing associated bone pain.

PAPA Syndrome

PAPA syndrome is another acne variant with systemic symptoms, marked by sterile pyogenic arthritis, pyoderma gangrenosum, and acne. Patients may also have a history of sterile cutaneous abscesses, inflammatory bowel disease, and pancytopenia following sulfa-containing medications.

It is an autoinflammatory disorder inherited in an autosomal dominant manner, resulting from mutations in the *CD2-binding protein-1* gene (also known as *PSTPIP1*), leading to increased IL-1 β production.

Successful treatments have been reported with infliximab and anakinra.

Acne Excoriée des Jeunes Filles

Acne excoriée des jeunes filles (French for "excoriated acne of young women") occurs primarily in young women who pick at their skin, although males can also be affected. Mild acne is often present, accompanied by extensive excoriations. Comedones and papules are systematically and neurotically excoriated, leaving crusted erosions that scar. The lesions excoriated are often minute.

This condition may indicate underlying depression, anxiety, obsessive-compulsive disorder, or a personality disorder. Antidepressants and psychotherapy can be helpful in management.

Acne Mechanica

Acneiform eruptions can develop after repetitive physical trauma to the skin, such as rubbing from clothing (belts, straps) or sports equipment (football

helmets, shoulder pads). Occlusion with adhesive tape can also cause acne mechanica. Obstruction of the pilosebaceous unit leads to comedo formation.

It presents as a well-defined, lichenified, hyperpigmented papule or plaque with interspersed comedones. A classic example is fiddler's neck, which occurs on the lateral neck from repetitive friction by a violin or viola chin rest.

Acne with Solid Facial Edema

A rare and disfiguring variant of acne vulgaris is acne with solid facial edema, also known as Morbihan disease. It presents with woody edema of the mid-third of the face, accompanied by erythema and acne lesions. Similar changes can occur in rosacea and Melkersson-Rosenthal syndrome.

The severity of edema may fluctuate, but spontaneous resolution does not occur. Oral antibiotics are typically ineffective.